

LABORATORY REPORT - FINAL



Name : Ms. SHRISTI SINGH	Gender : Female	Lab ID : 60666701715
Age : 22 Years	Mob. No. : 8217682108	Pt. ID : 9701604
B2B : RAVI LAB CHIKKAMAGALURU	Ref. By : MGH CKM	
Reg Date and Time : 26-Jun-2026 11:57	Report Date and Time :	Ref Id1 :
Sample Received at : KA-LLM-Chikkamagaluru		Ref Id2 :
Sample Collected at :		

Out of Reference Range Result(s) Summary

Test Name	Result Value	Unit	Reference Range
Testosterone	62.01	ng/dL	13.84 - 53.35

Abnormal Result(s) Summary End

Note:

This summary highlights abnormal and trend results extracted from the report for quick reference.
It is for informational purposes only and does not require a doctor's signature.

Note:(LL-VeryLow,L-Low,H-High,HH-VeryHigh,A-Abnormal,HC- High Critical,LC-Low Critical,C-Critical)

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Reg Date and Time : 26-Jun-2026 11:57 **Report Date and Time** : 27-Jun-2026 12:50 **Ref Id1** :
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Sample Collected at :

TEST	RESULTS	UNIT	REFERENCE RANGE	METHOD
Testosterone	H 62.01	ng/dL	13.84 - 53.35	CMIA
Leutinizing Hormone	9.48	mIU/mL	Follicular Phase : 1.7-15 Ovulation Phase : 21.9 - 56.6 Luteal Phase : 0.62 - 16.3 Post menopause : 14.2 - 52.3	CMIA

INTERPRETATIONS:

Useful as an adjunct in the evaluation of menstrual irregularities, Evaluating patients with suspected hypogonadism, Predicting ovulation, Evaluating infertility, Diagnosing pituitary disorders.

In both males and females, primary hypogonadism results in an elevation of basal follicle-stimulating hormone (FSH) and luteinizing hormone (LH) levels.

Postmenopausal LH levels are generally >40 IU/L. (Note: FSH is the preferred test to confirm menopausal status.)

FSH and LH are generally elevated in:

Primary gonadal failure, Complete testicular feminization syndrome, Precocious puberty (either idiopathic or secondary to a central nervous system lesion), Menopause,

Primary ovarian hypodysfunction in females, Polycystic ovary disease in females, Primary hypogonadism in males

LH is decreased in Primary ovarian hyperfunction in females, Primary hypergonadism in males.

FSH and LH are both decreased in failure of the pituitary or hypothalamus

CAUTIONS:

No clinically significant cross-reactivity has been demonstrated with follicle-stimulating hormone, thyroid stimulating hormone, or human chorionic gonadotropin.

Some patients who have been exposed to animal antigens, either in the environment or as part of treatment or imaging procedures, may have circulating antianimal antibodies present. These antibodies may interfere with the assay reagents to produce unreliable results

Note:(LL-VeryLow,L-Low,H-High,HH-VeryHigh,A-Abnormal,HC- High Critical,LC-Low Critical,C-Critical)

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Jayanthi P.R.

Dr.Jayanthi P R
Consultant Biochemist
KMC. NO.: 42548

Prajwal A

Dr Prajwal A
Consultant Biochemist
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Printed On: 27-Jun-2026 13:25

Neuberg Anand Reference Laboratory - A Unit of Neuberg Diagnostics Private Limited

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Follicle Stimulating Hormone 7.39 mIU/mL
Follicular 11.6 - 1.98 - CMIA
midcycle 23.4 - 5.14 -
Luteal phase 9.58 - 1.38 -
Post menopausal 131 - 21.5 -

INTERPRETATIONS:

Useful for an adjunct in the evaluation of menstrual irregularities, Evaluating patients with suspected hypogonadism, Predicting ovulation, Evaluating infertility & Diagnosing pituitary disorders.

In both males and females, primary hypogonadism results in an elevation of basal follicle-stimulating hormone (FSH) and luteinizing hormone (LH) levels.

FSH and LH are generally elevated in:

-Primary gonadal failure, Complete testicular feminization syndrome, Precocious puberty (either idiopathic or secondary to a central nervous system lesion), Menopause (postmenopausal FSH levels are generally >40 IU/L), Primary ovarian hypofunction in females, Primary hypogonadism in males.

Normal or decreased FSH in:

-Polycystic ovary disease in females

FSH and LH are both decreased in failure of the pituitary or hypothalamus.

CAUTIONS:

Some patients who have been exposed to animal antigens, either in the environment or as part of treatment or imaging procedures, may have circulating antianimal antibodies present. These antibodies may interfere with the assay reagents to produce unreliable results.

Specimen: Serum

Coll. Date: 26-Jun-2026 11:57

Rec. Date: 26-Jun-2026 11:57

Approved By: Dr. Jayanthi P R

All the above examination performed at Neuberg Anand Reference Laboratory and the results have been reviewed and released by its authorised signatory.

Note: (LL-VeryLow, L-Low, H-High, HH-VeryHigh, A-Abnormal, HC- High Critical, LC-Low Critical, C-Critical)

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TEST	RESULTS	UNIT	REFERENCE RANGE	METHOD
Ferritin	40	ng/mL	22-439	CMIA
Specimen:Serum	Coll. Date:26-Jun-2026 11:57	Rec. Date:26-Jun-2026 11:57		

Approved By: Dr Prajwal A

All the above examinations have been performed at Neuberg Anand Reference Laboratory **KA-Shivajinagar** and the results have been reviewed and released by its authorised signatory.

----- End Of Report -----

For test performed on specimens received or collected from non-NARL locations, it is presumed that the specimen belongs to the patient named or identified as labeled on the container/test request and such verification has been carried out at the point generation of the said specimen by the sender. NARL will be responsible Only for the analytical part of test carried out. All other responsibility will be of referring Laboratory.

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